

BRN Case Management CE Package - Draft / Pending BRN CEP Approval

Chronic Disease, Geriatric, and High-Risk Patient Management

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"Pending California Board of Registered Nursing Continuing Education Provider approval. Do not advertise, issue certificates, or represent this course as approved until a BRN CEP number has been issued."

This document follows a CCCCCO-style documentation organization only (structured overview, objectives, content outline, activities, facilitator notes, assessment mapping, compliance support). No CCCCCO content is copied; all content is original and customized for California BRN Case Management CE.

Module At a Glance

Field	Value
Module ID	CM-M09
Module title	Chronic Disease, Geriatric, and High-Risk Patient Management
Parent course	RN Case Management in Home Health: Care Coordination, Documentation, Patient Advocacy, and Safe Transitions of Care
Contact hours	3 (draft design math)
Estimated learner minutes	170 (≥ 150 floor)
Direct/indirect linkage	Direct patient/client care (scientific knowledge)
Approval status	Draft / Pending BRN CEP Approval

BRN Relevance Rationale

Applies clinical/scientific knowledge to manage high-risk populations and reduce decompensation.

Module Description

Strengthens risk-stratification, chronic-disease management priorities, geriatric-syndrome awareness, and monitoring plans with escalation triggers. Educational content only; not medical advice.

Measurable Learning Objectives

By the end of this module the learner will be able to:

- 1 Apply risk-stratification to identify high-risk patients in a panel.
- 2 Describe nursing management priorities for common chronic conditions in care management.
- 3 Identify geriatric-specific risks (falls, polypharmacy, cognition) and nursing mitigations.
- 4 Build a monitoring plan with escalation triggers.

Required Readings / Learning Content Sections

Reading 9.1 - Risk stratification.

Reading 9.2 - Chronic-disease management priorities.

Reading 9.3 - Geriatric syndromes and polypharmacy.

Reading 9.4 - Monitoring plans and escalation triggers.

Detailed Lesson Outline

Lesson ID	Lesson	Est. minutes
CM-M09-L1	Risk stratification	40
CM-M09-L2	Chronic-disease management priorities	40
CM-M09-L3	Geriatric syndromes & polypharmacy	35
CM-M09-L4	Monitoring plans & triggers	35
CM-M09-KC	Knowledge check + risk-stratification worksheet	20

Total estimated instructional minutes: **170** (meets the ≥ 150 -minute / 3-contact-hour floor).

Key Terms

Term	Definition
Risk stratification	Classifying patients by risk to target resources.
Polypharmacy	Use of multiple medications, raising interaction/adverse-event risk.
Geriatric syndrome	Multifactorial conditions common in older adults (falls, delirium).
Escalation trigger	A predefined change that prompts a higher level of action.
Decompensation	Functional/clinical decline as a condition worsens.

Fictional / De-identified Case Scenario

All clinical content is fictional and de-identified. No PHI. A fictional 80-year-old with heart failure, diabetes, and mild cognitive impairment takes nine medications and had one recent fall.

Learner Activities

- Read Sections 9.1-9.4.
- Stratify the client's risk level with justification.
- Identify polypharmacy and fall risks with nursing mitigations.
- Build a monitoring plan with ≥ 3 triggers and complete Knowledge Check 9.

Documentation / Decision-Making Exercise

Complete a Risk-Stratification + Monitoring Worksheet for the scenario, including at least three escalation triggers tied to specific findings.

Knowledge Check Blueprint

10 items, unlimited attempts, immediate feedback, no full key, $\geq 80\%$ to complete.

Assessment Items (Learner-Facing Stems)

KC9-Q1 (Multi-select). Which factors raise this geriatric client's risk profile? (select all)

- Polypharmacy (9 meds)
- Recent fall
- Cognitive impairment
- A single stable condition

KC9-Q2 (MCQ). An appropriate escalation trigger in a heart-failure monitoring plan is:

- All weight gain of ≥ 3 lb in a day / 5 lb in a week
- Any change in TV channel
- Normal daily walk
- Stable weight

KC9-Q3 (True/False). Polypharmacy increases the risk of adverse drug events and warrants medication review.

- True
- False

LMS Completion Criteria

- Lesson pages viewed (activity completion: require view).
- M09 knowledge check passed at $\geq 80\%$ (require passing grade).
- Documentation/decision exercise submitted (require submission).

Evidence Generated by Learner Completion

- Activity-completion timestamps (time-on-task evidence).
- Knowledge-check score and attempt log.
- Submitted exercise artifact (retained ≥ 4 years; no PHI).

Moodle Activity Recommendations

- Peer
- Book
- Quiz (KC9)
- Assignment (worksheet)

Accessibility Considerations

- Captions + transcripts for any media; semantic headings; alt text on diagrams.
- Sufficient color contrast; no color-only meaning; keyboard-navigable activities.
- Accessible downloadable templates (tagged PDF + DOCX); extended-time accommodation supported.
- Target WCAG 2.1 AA on the built course.

PHI / Privacy Guardrails

- All cases fictional and de-identified; no real patient/resident/family/facility data.
- Functional clinical profile only; avoid color-only risk tiers (use labels).
- Submission reminders and (for capstone) attestation + manual PHI review.

SME Review Flags

bullet [SME REVIEW REQUIRED]] Confirm clinical content currency; mark as education, not medical advice or standing orders.

Compliance Review Flags

bullet [COMPLIANCE REVIEW]] Fictional clinical profile only; avoid color-only risk tiers (use labels).

Internal Answer Key Appendix (ADMIN / FACULTY ONLY - DO NOT SHOW LEARNERS)

Item ID	Type	Correct answer	Rationale	Remediation
KC9-Q1	Multi-select	A, B, C	A-C are high-risk geriatric markers; D is comparatively lower risk.	Review Reading 9.1/9.3.
KC9-Q2	MCQ	A	Rapid weight gain can signal fluid overload requiring escalation.	Review Reading 9.4.
KC9-Q3	True/False	True	Multiple medications raise interaction/adverse-event risk.	Review Reading 9.3.

Debrief / Remediation Guidance

Show how stratification drives monitoring intensity and triggers, and how polypharmacy review prevents harm. Failed attempts route to Reading 9.1/9.3/9.4. Reinforce content is educational, not medical advice.

Application-Packet Defensibility Notes

Application of scientific/clinical knowledge to high-risk patients is core direct patient care and strongly BRN-relevant.

Program-structure note: This module is part of the 30-contact-hour, 10-module RN case management program (10 modules x 3 contact hours; 50 instructional minutes = 1 contact hour; minimum 1,500 instructional minutes, 1,680 design minutes). The official BRN contact-hour map and transfer worksheet are aligned to this 30-contact-hour structure. Any earlier 4-contact-hour representative course is superseded and is not part of the current BRN CEP signer-review package. [[SME REVIEW REQUIRED BEFORE SUBMISSION]]

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